Gonzaga Dental Clinic Management System

UI/UX walkthrough — every screen, with notes on what it does and why it is built that way.

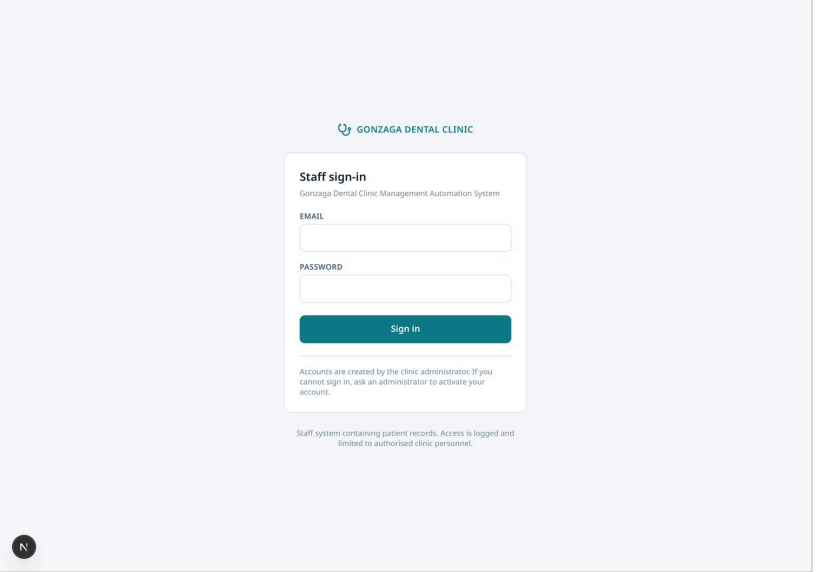
Scope patient records, appointments, consent, treatments, staff and audit
Roles dentist, secretary, admin — patients are records, not user accounts
Screens documented 19
Data in these images fabricated samples, generated locally

Preview build. These screenshots come from a preview mode with no database attached. Every record shown is invented, the red banner is always visible, and the system is read-only: sign-in, saving, and permission enforcement are not exercised here. Preview mode must be switched off before any real patient data is entered.

01 / 19 Sign in

> The only page reachable without a session.

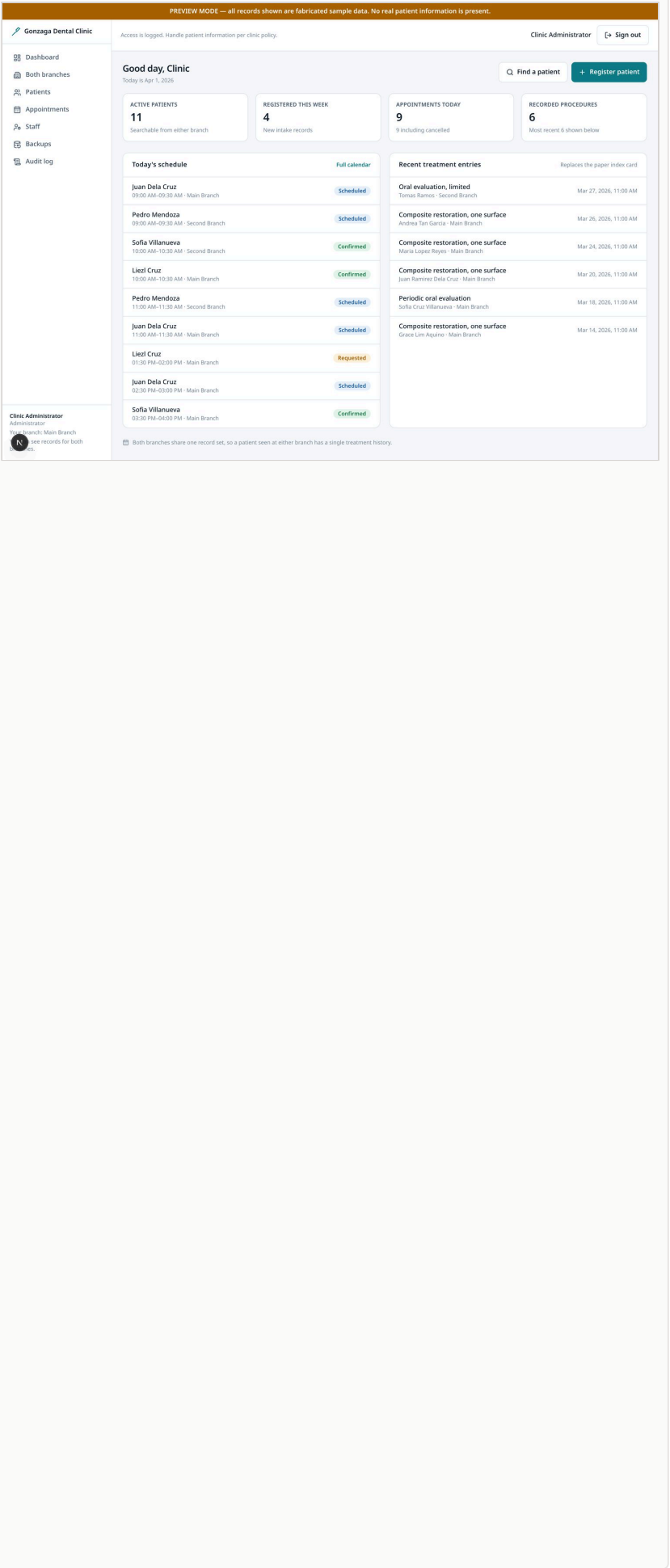
- Email and password only. There is no patient login, because patients are records, not accounts.
- A valid password is not enough: the server layout re-checks that the staff row is still active, so a deactivated account cannot reach the app even with a valid password.
- The 'next' parameter is validated so it can only be a local path, closing off open-redirect abuse.
- Error text is deliberately vague. It never reveals whether an email has an account, which would leak staff existence to an attacker.



Dashboard

> The landing page after sign-in: what needs attention today.

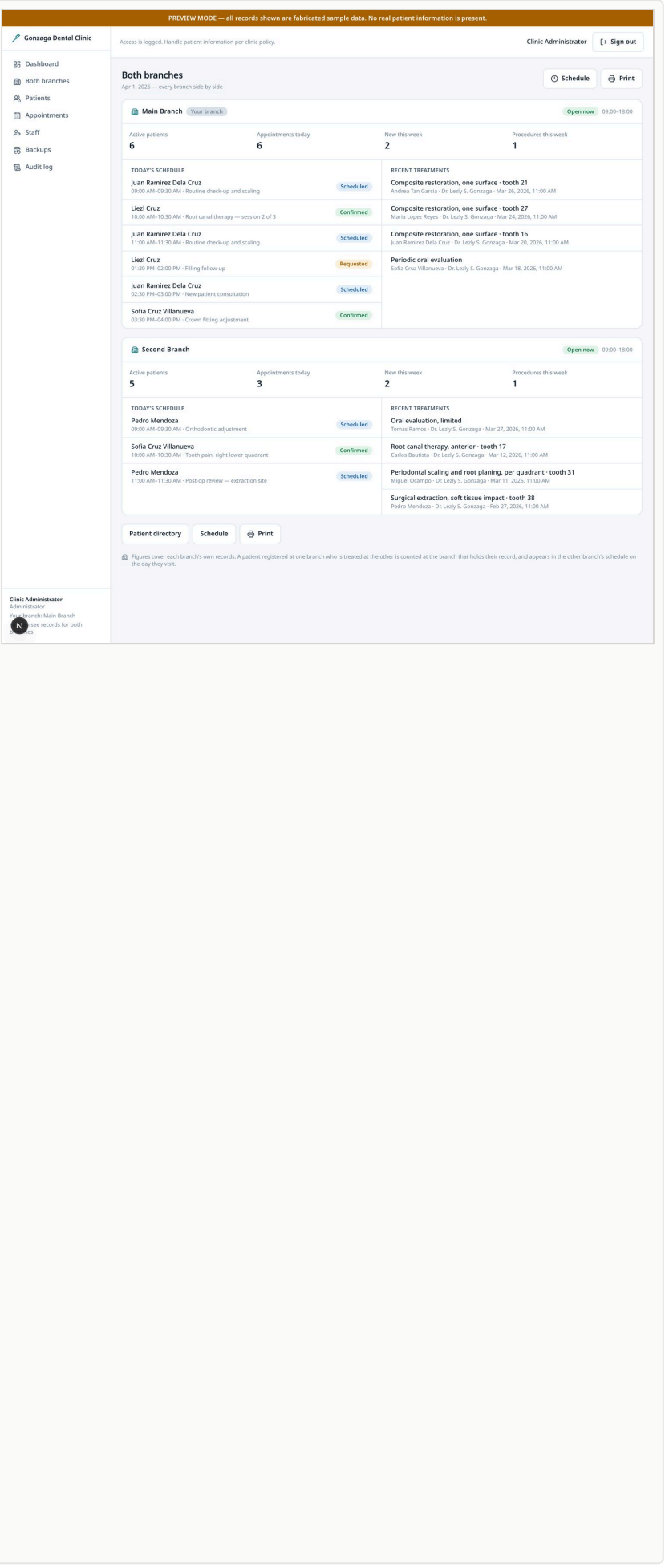
- Today's appointment list first, because that is the question the secretary asks on arrival.
- Counts for patients, appointments and treatments give orientation without needing to open each list.
- Recent treatment entries surface cross-branch clinical activity.
- Times render in clinic time (Asia/Manila) regardless of the server's timezone.



Both branches

> The dentist's view of the clinic: two branches, side by side.

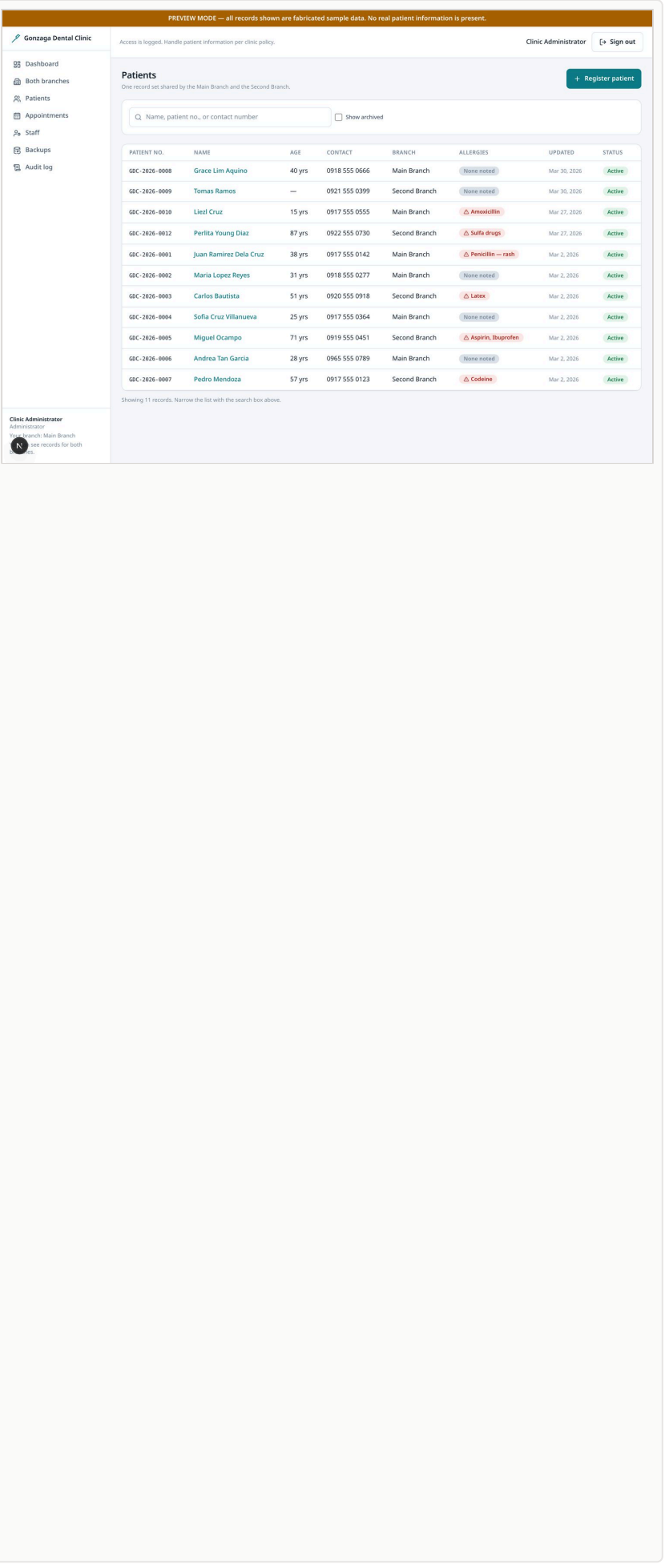
- Each branch gets its own block with its own figures. Merging them into one list would hide the thing that matters when you own two sites: whether one of them is behind.
- Per branch: active patients, appointments today, new registrations this week, and procedures recorded this week.
- Today's schedule is split by branch rather than interleaved, so the load at each site is visible at a glance.
- Open/closed status uses each branch's own hours, so a branch that has already closed reads differently from one still trading.
- Visible to dentists and admins. A secretary is scoped to a single branch by the database itself, so this comparison is not something they could reconstruct from the patient list.
- Reads on the phone as well, with the per-branch figures stacking into a single column.



Patient directory

> Every patient, searchable, with the duplicate warning built in.

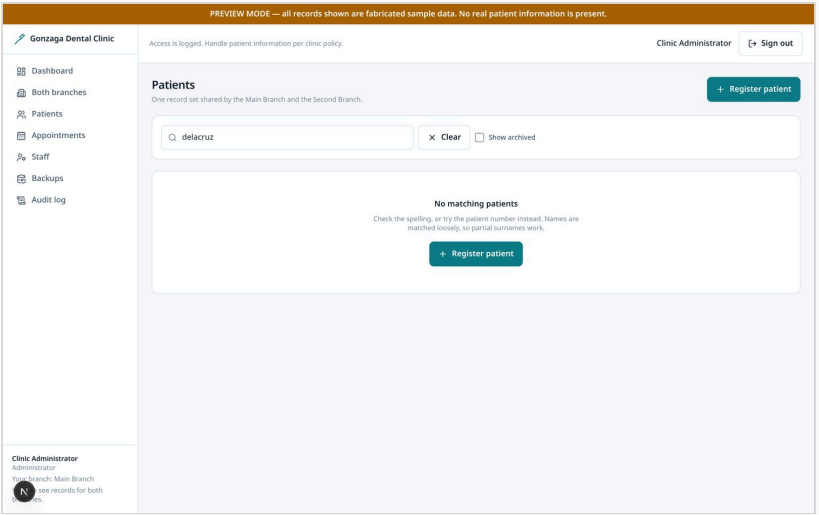
- Search filters from the URL, so a filtered view can be bookmarked or shared with a colleague.
- Patient number, name, age and branch are shown together because staff identify people by any of them.
- New registrations are checked against likely duplicates by name and birthdate. Similarity is surfaced for a human to judge, never auto-merged, because merging patient records is unsafe to automate.



Search results

> The same directory after a query, to show filtering end to end.

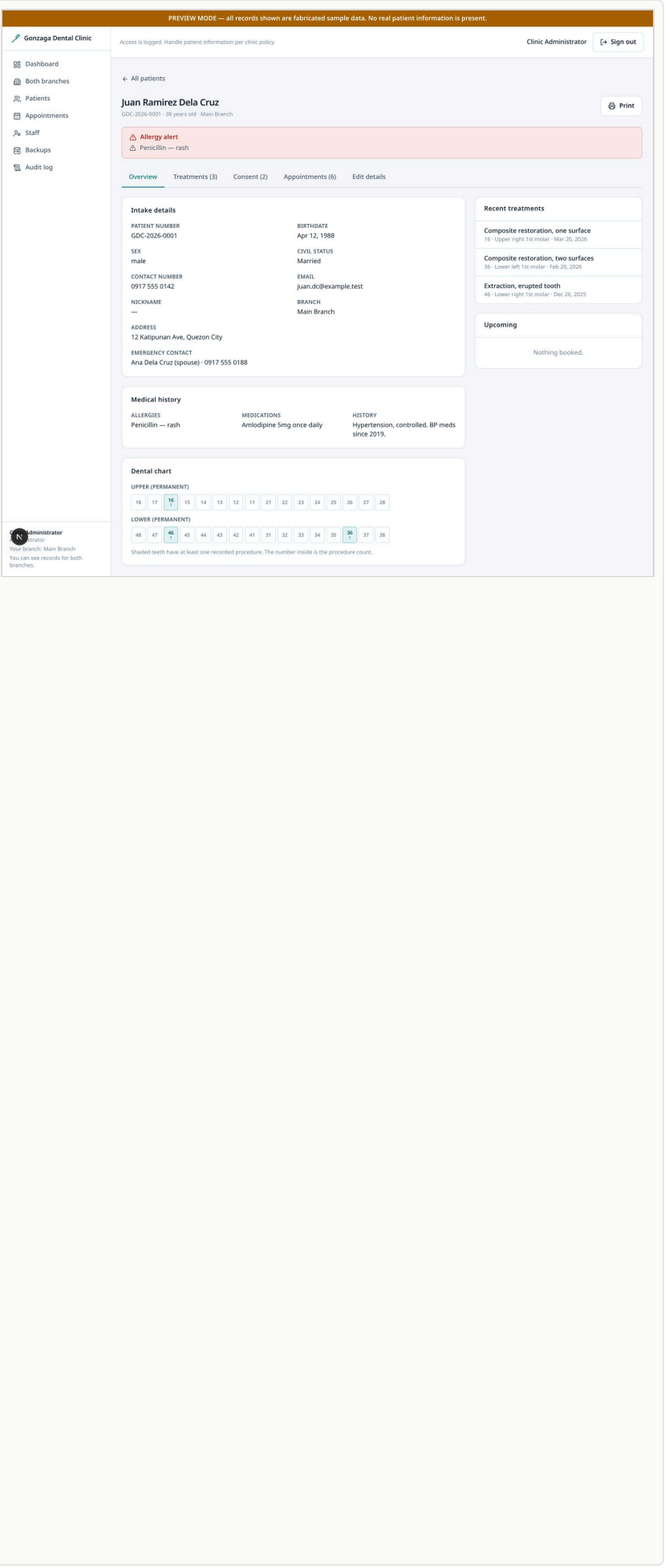
- Results update without a full page load and the query stays in the address bar.
- An archived patient is hidden by default; the toggle includes them for records that need correcting.



Patient record: overview

> Demographics, medical context and upcoming visits for one patient.

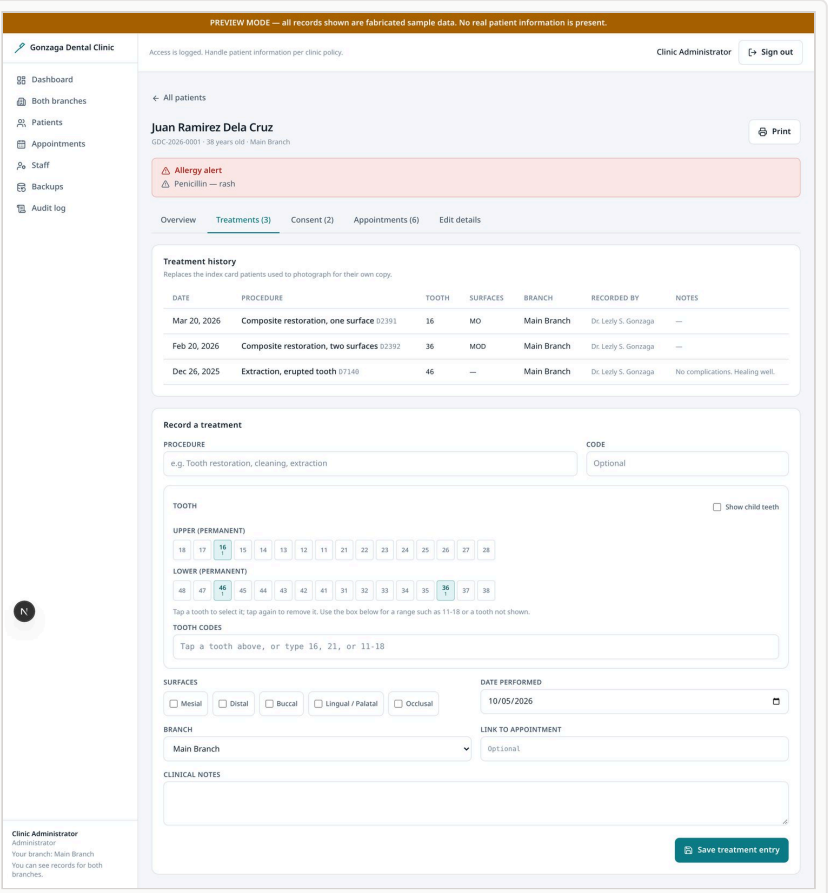
- Allergies are placed at the top, not buried in a tab, because an unnoticed allergy is a clinical risk.
- Demographics are read-only here and edited through a separate, deliberate flow.
- The tab bar (overview, treatments, consent, appointments, edit) is the whole record; nothing important is more than one click away.



Patient record: treatments

> The clinical history that used to live on a paper index card.

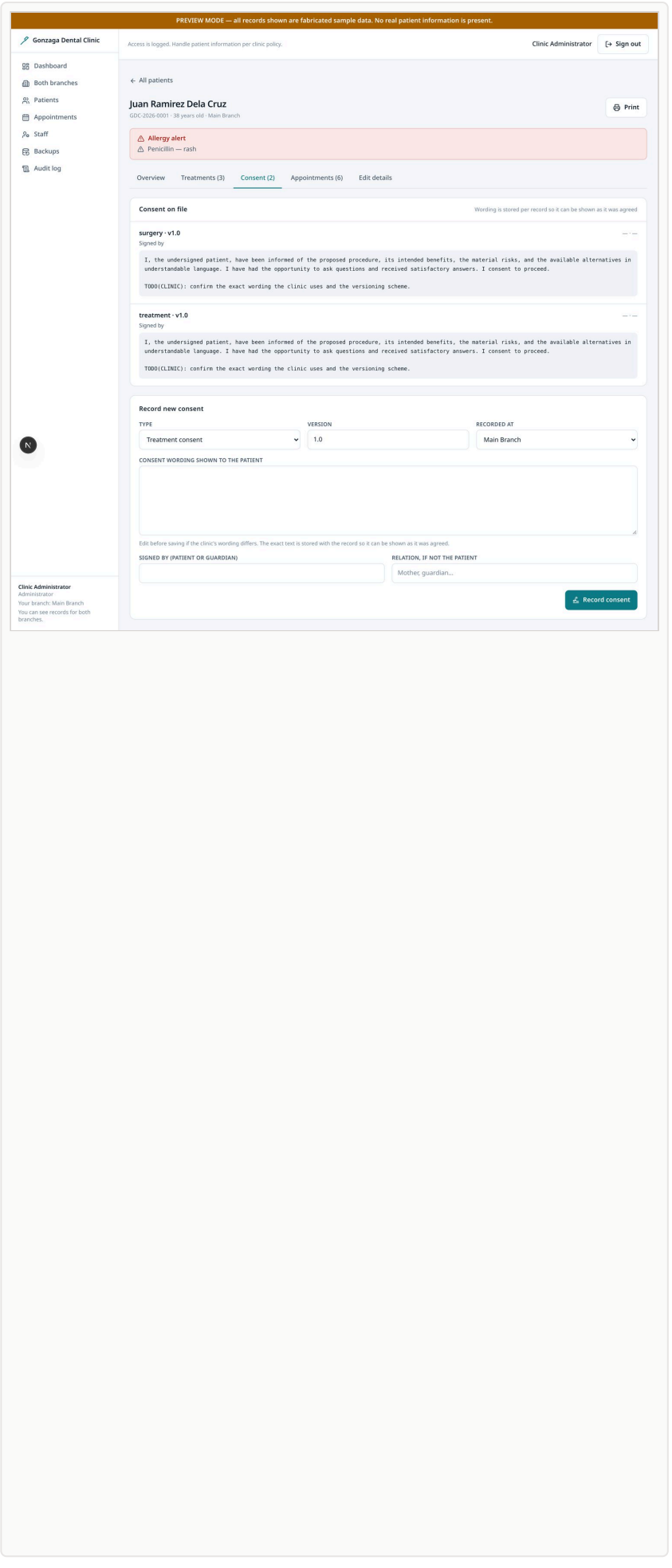
- The tooth is chosen by tapping it on the chart rather than typing a two-digit code. A dentist identifies a tooth by where it sits; asking for a number is the slowest, most error-prone way to record it.
- Teeth already treated for this patient are shaded with their procedure count, so prior work on the tooth is visible while charting rather than after.
- The chart and the text box are two views of one value, so a tapped tooth and a typed range can never disagree about what gets saved. Child teeth are one checkbox away.
- Charting follows the FDI two-digit notation, so a tooth is named the same way the clinic already names it.
- Each entry records surfaces, procedure and who performed it, which is what an audit or a complaint review needs.
- Print produces a clean chart for the physical file, with navigation and controls hidden.



Patient record: consent

> Consent captured per patient, versioned over time.

- A consent record stores the exact version the patient agreed to, not just a checkbox.
- History is retained so a later dispute can be answered with what was actually signed at the time.
- Consent wording is a clinic decision and is deliberately not invented here.



Patient record: appointments

> This patient's appointment history.

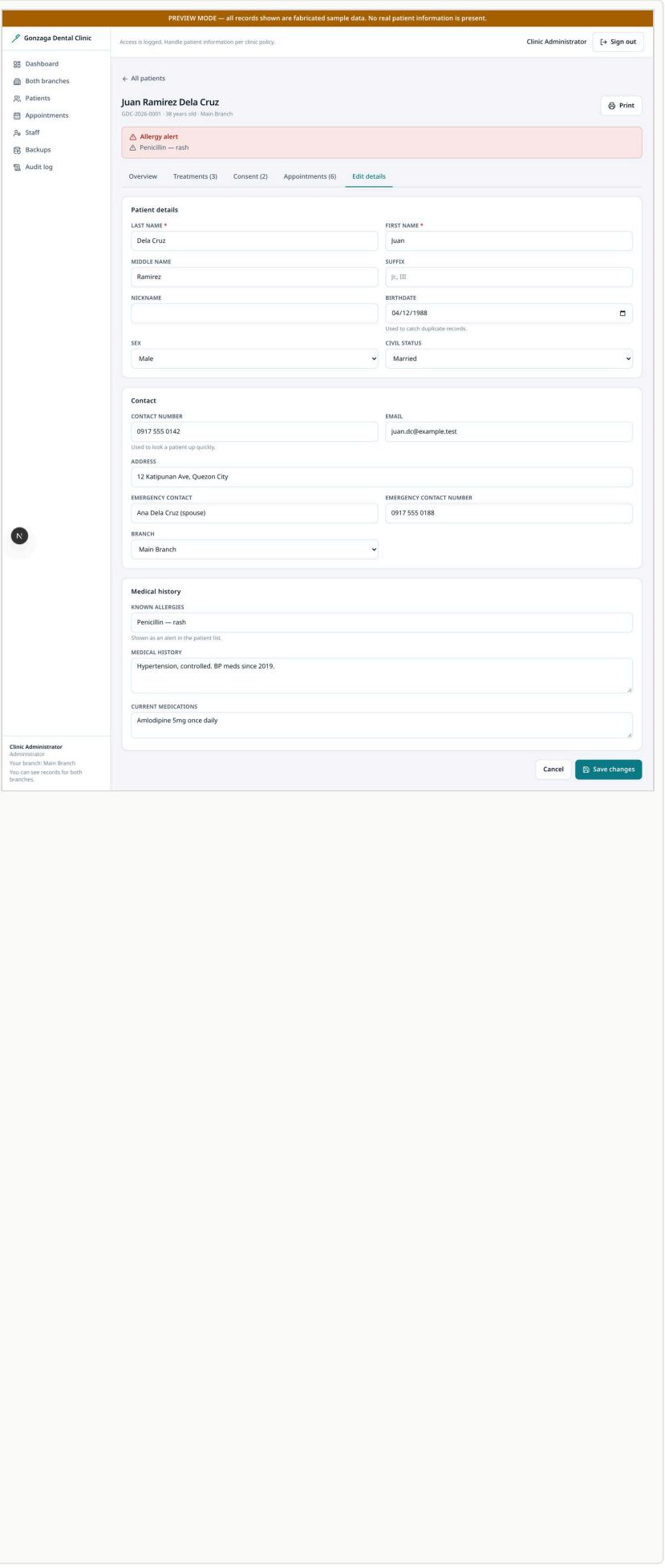
- Past and upcoming are separated so the two are never confused.
- Overlapping appointments for the same patient are rejected by the database, not just by the form, so a race between two secretaries cannot double-book.



Patient record: edit

> Editing an existing record rather than creating a new one.

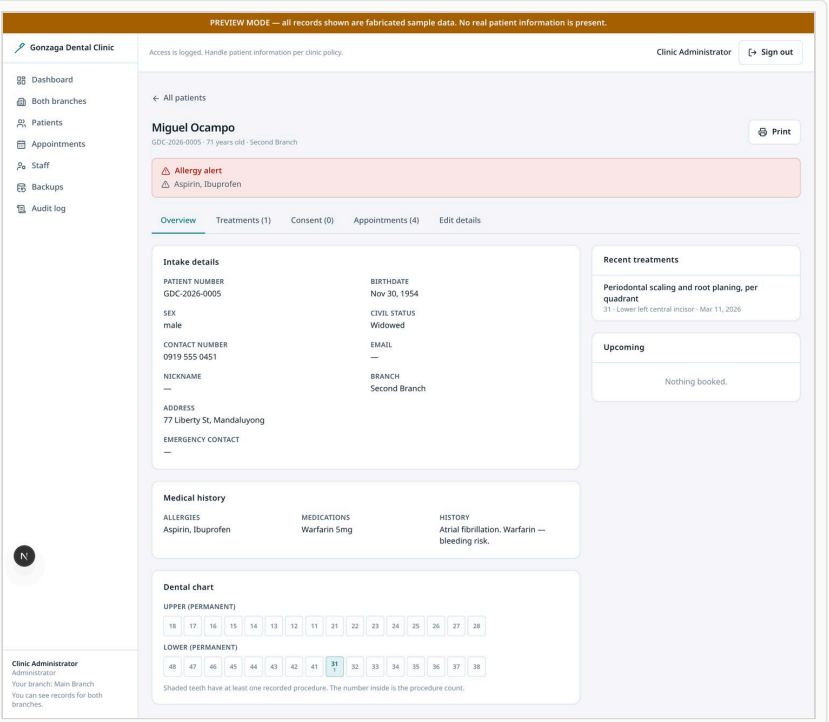
- The form is pre-filled from the stored record, so an edit never silently blanks a field.
- Edits go through the same validation as registration, so a correction cannot create a record that registration would have refused.



High-risk record: allergy alert

> The same overview, chosen to show how risk is escalated.

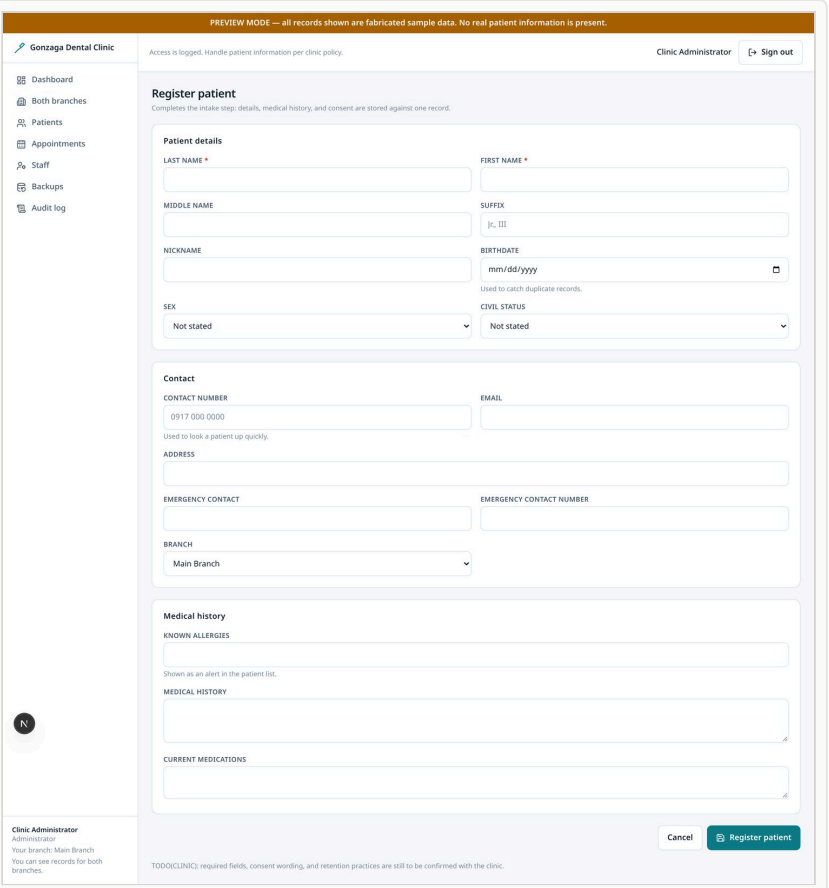
- A patient with a recorded allergy is flagged prominently rather than left for the reader to notice.
- Escalation is visual and immediate because the cost of missing it is a medical incident.



Register patient

> Digital intake, replacing the paper front-desk form.

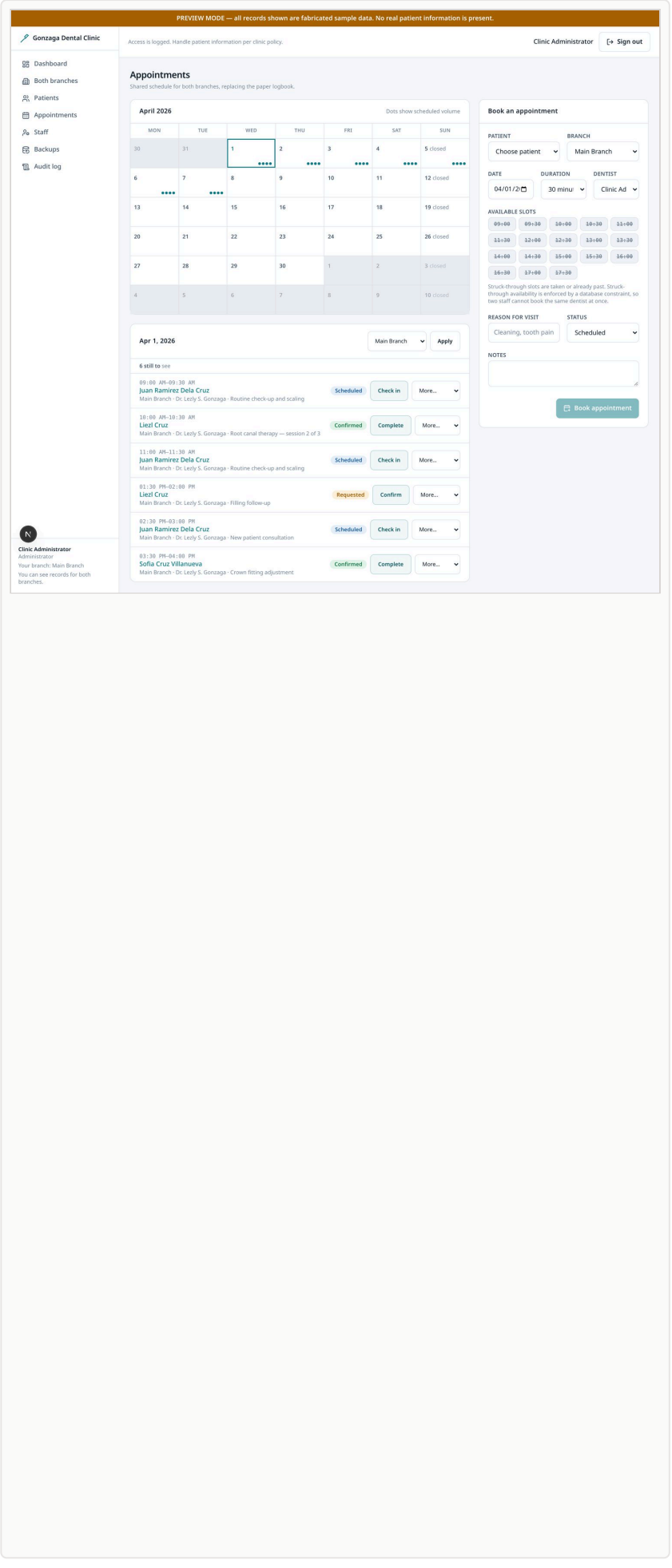
- Field-level validation messages appear next to the field that failed.
- A birthdate is optional. Blank means unknown, which is different from a wrong date, so it is never guessed.
- On a likely duplicate match the form pauses for explicit acknowledgement rather than creating a second record silently.



Appointments

> The shared clinic schedule across branches.

- A month grid with per-day volume dots gives an overview before drilling into a day.
- The day view can be filtered by branch, and is labelled by branch to keep 'your branch' distinct from the clinic-wide record.
- The day list opens with a count of how many patients are still to be seen, so progress is readable without counting rows.
- The transition a secretary repeats all morning — check in, then complete — is a single labelled button on each row rather than an item in a dropdown. Anything else stays available under 'More'.
- Only the row being saved is disabled while it saves, so one slow request does not freeze the rest of the waiting room.
- The branch filter and its Apply control are both full-size touch targets for tablet use at the front desk.

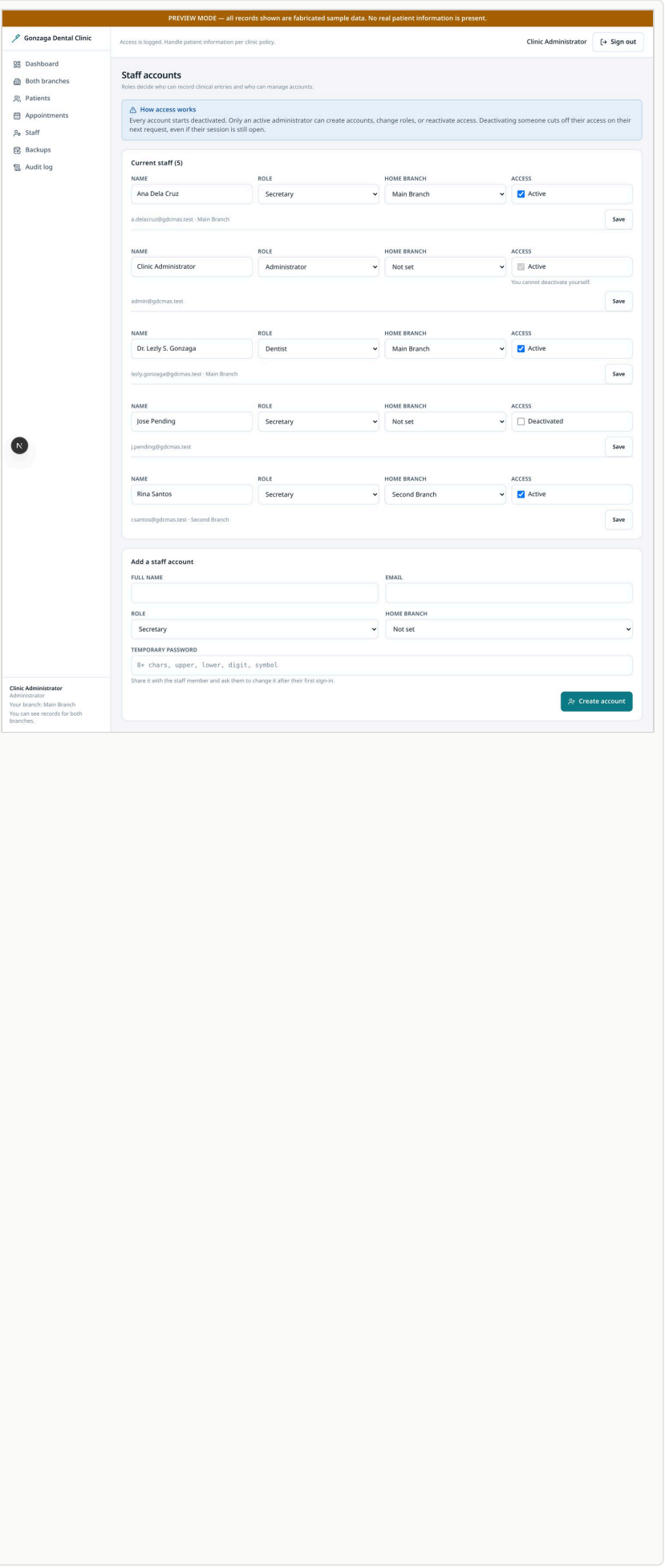


Staff

ADMIN ONLY

> Staff accounts and roles. Admin only.

- Three roles: dentist, secretary, admin. Permissions are enforced by row-level security in the database, not by hiding buttons.
- The most recently created accounts are listed first so a new hire is easy to verify.
- An admin cannot deactivate their own account, which prevents locking the clinic out of its own system.

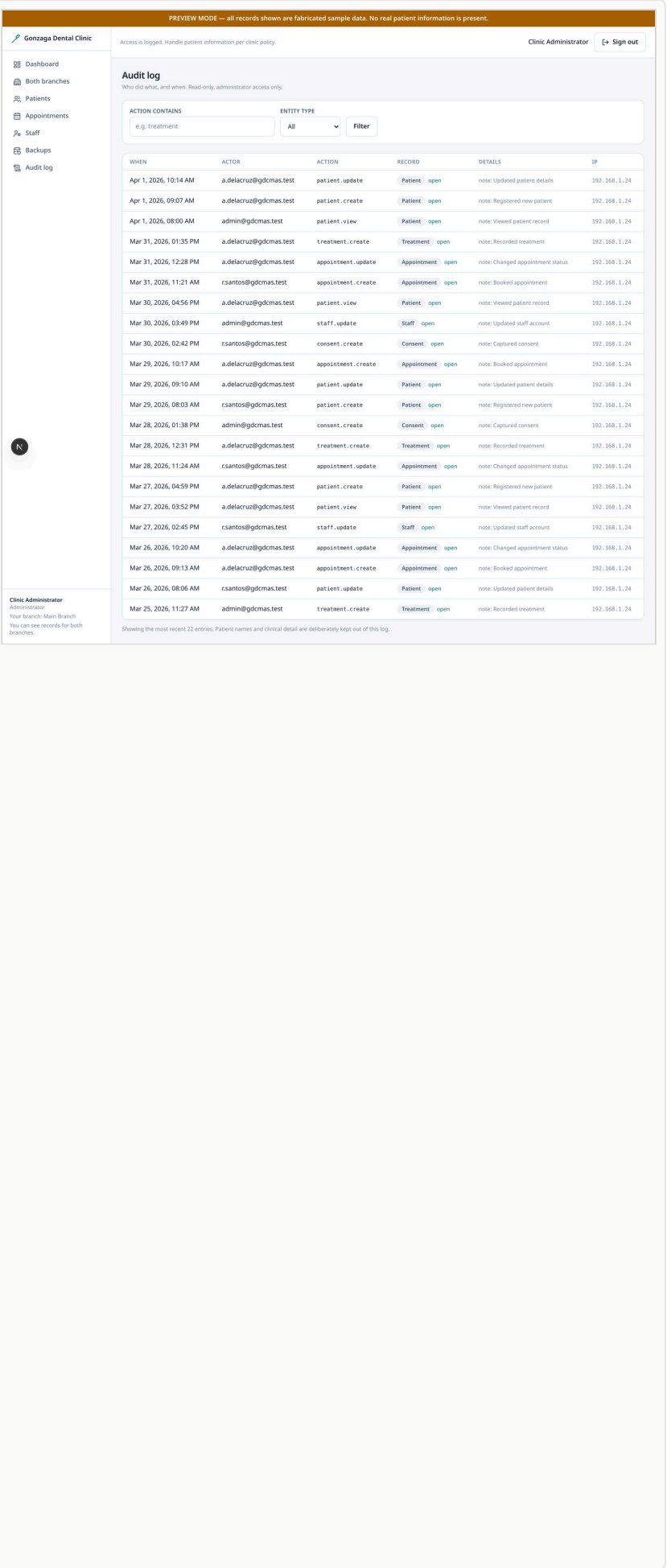


Audit log

ADMIN ONLY

> Append-only record of access and changes. Admin only.

- Every read and write of patient data is recorded, because the system holds identifiable health information.
- Rows link to the record they describe, turning a log line into a starting point for investigation.
- The log is append-only: there is no edit or delete affordance anywhere in the interface.

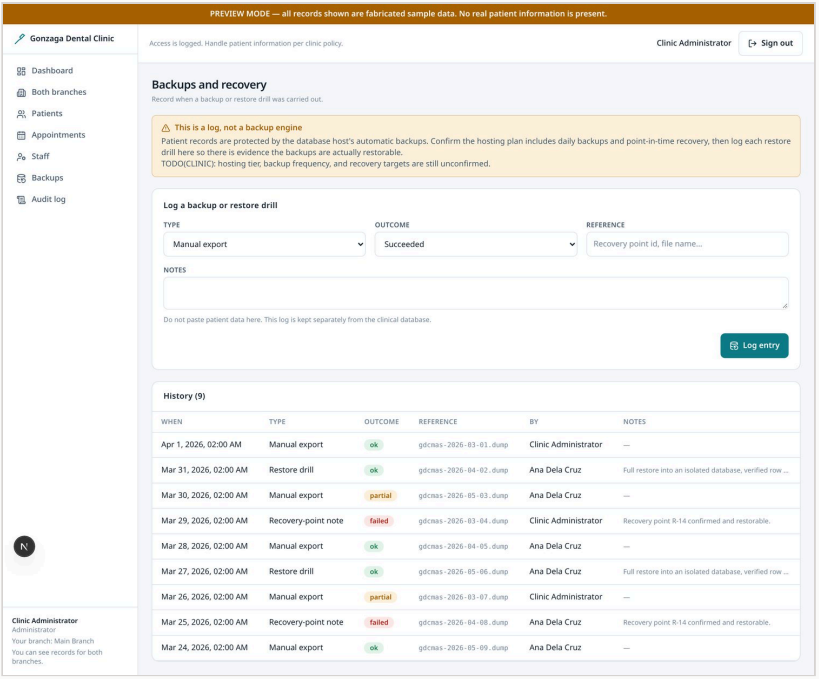


Backups

ADMIN ONLY

> Backup and export activity. Admin only.

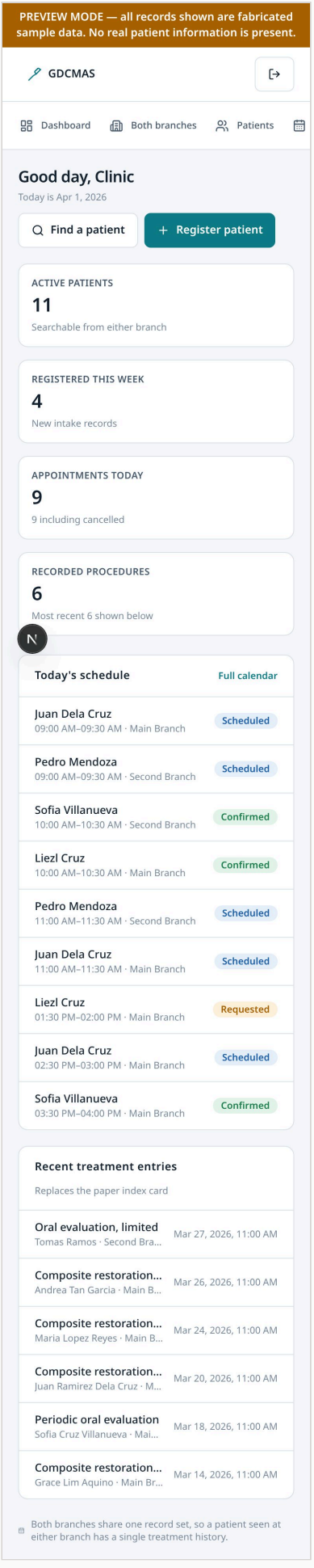
- A clinic holding patient records needs a recoverable copy and a way to take one deliberately.
- The log records what was exported, by whom and when, so leaving the building with a backup is itself traceable.
- Real scheduled backups are a deployment decision and are not automated by this application.



Mobile: dashboard

The dashboard at phone width.

- The sidebar collapses to a horizontal scrolling nav so navigation survives a narrow screen without hiding it behind a menu.
- Interactive controls are at least 44px tall, the minimum comfortable touch target, because front-desk staff work on phones.



Mobile: patient directory

The patient list at phone width.

- The table remains readable by scrolling horizontally within the table, so columns are never silently dropped.
- Patient names are comfortably tappable rather than bare inline text.



Both branches on a phone

> The same cross-branch comparison, on the screen most staff actually carry.

- Each branch becomes its own stacked block, so the counts, schedule, and recent treatments are read by scrolling rather than side by side.
- The branch name and open/closed badge stay at the top of each block, which is the part that has to be legible at a glance before opening anything else.

